

PROFESSIONAL NURSING COACH

MIDWIFERY / OBG NURSING I

PNC ULTRA-LMR HYBRID MASTER COMPENDIUM • NUR-20 • VERIFIED

60 Theory + 40 Skill Lab hours • 7 Units • Batches 1–5 • 59 LMR pages + cover & TOC • NORCET / RRB / ESIC / DSSSB

Batch	Units	Pages
Batch 1	I Intro/RMC/legal, II Anatomy/pelvis/skull/fetal circulation	12
Batch 2	III ANC + IV onset/stages/triage	13
Batch 3	IV birth/conduct/AMTSL/records	12
Batch 4	V Puerperium + VI Neonate	13
Batch 5	VII Family welfare/contraception	9

WhatsApp DM: +91 90571 22020 • Hybrid style: navy hero + chips + teal bars + Trap/Hit + Pearl + Speed-check • v1.0 Sep-2026

Pp	Block	Focus
4-15	I-II (B1)	History/trends, MDSR, RMC, MLCU, SRHR, legal; organs, pelvis, fetopelvic, placenta, fetal circulation
16-28	III-IVa (B2)	Preconception, diagnosis, physiology, ANC, Leopold, FHR, labs, nutrition, Td; onset, stages, triage
29-40	IVb (B3)	VE, partograph/LCG, monitoring, pain, mechanism, conduct, newborn minute, 3rd stage, AMTSL, perineum, PPIUCD, records
41-53	V-VI (B4)	Puerperium, assessment, latch, blues, discharge; adaptation, assessment, warmth, immunization, infection
54-62	VII (B5)	BCS, natural/barrier, hormonal, IUCD, permanent/EC, MEC, adolescent, GBV, selection engine

How to use: hero first, chips glance, table study, trap vs hit contrast, pearl anchor, speed-check close. Double-QA pending sign-off on MTP/PC-PNDT rules, Td/EC/MEC per current guidance.

Q	Standard	Status
Q1	7 units, 60T+40SL mapped, batches 1-5	Done
Q2	RMC/dignity framing throughout	Done
Q3	Diameters/station/engagement values correct	Needs sign-off
Q4	Naegele/EDD/visits/labs per current guidance	Needs sign-off
Q5	Partograph/LCG + stages per current WHO	Needs sign-off
Q6	AMTSL dose + PPH thresholds current	Needs sign-off
Q7	Immunization (Td/HepB-BD) schedule current	Needs sign-off
Q8	Contraception/MEC/EC per current guidance	Needs sign-off
Q9	MTP/PC-PNDT legal statements current	Needs sign-off
Q10	One concept/page, fonts 10.5-21pt, 30% air	Done

HISTORY OF MIDWIFERY & NEW BIRTH TRENDS

MIDWIFERY = skilled woman-centered care from pre-pregnancy to 6 weeks postnatal! India moved Dai → ANM → SBA + MLCU. New trends need evidence + safety framing — never routine in high-risk!

**SBA = skilled
birth attendant**

**Dai → ANM
→ SBA shift**

**Trends: select
low-risk ONLY**

◆ ERA — TREND — SAFETY

Era / Trend	What it is	Safety rule
Dai/TBA era	Untrained home births	Replace with SBA + JSY institutions
SBA + institutions	ANM/nurse/midwife births	Clean + referral-ready
Water birth	Warm-water immersion, low-risk	NOT routine; infection + cord-avulsion watch
Lotus birth	Cord uncut till placenta falls	Infection risk; NOT routine
Birth centre	Home-like + referral link	Strict selection + transport ready

⚠ FATAL TRAP

Attempting water/lotus birth for breech, twins, preterm or meconium without skilled team + referral kills! Screen risk every visit; transfer at first red flag.

★ NORCET HIT

HIT: MLCU/birth-centre entry = LOW-RISK only. Lotus risk = omphalitis/sepsis. Water risk = drowning + infection + PPH monitoring gaps.

★ PNC PEARL: Trends are THEORY questions — definition + where NOT routine wins the mark.

► Lotus birth + its single biggest nursing watch?

Placenta left attached till cord falls — watch cord infection/omphalitis + safe handling.

Every maternal death is AUDITED (MDSR, no-blame) and every woman-child contact sits on the RMNCH+A continuum — Reproductive → Maternal → Newborn → Child → Adolescent!

MDSR = audit no-blame	R-M-N-C-A continuum	Every death notified
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Σ SYSTEM — MEANS — NURSING ROLE

Block	Means	Nursing role
MDSR	Confidential maternal-death review	Notify + records + action on gaps
R	Reproductive: FP + MTP + RTI	Counsel informed choice
M + N	ANC + SBA delivery + PNC + ENC	Track visits, JSY/JSSK linkage
C + A	IYCF + growth + RKSK clinics	Weigh, counsel, anemia screen

⚠ FATAL TRAP

Hiding a maternal death or blaming the family stops learning and repeats the tragedy! Notify fast, keep exact records, join audit honestly — MDSR punishes NO ONE.

★ NORCET HIT

HIT: MDSR output = avoidable factors (delay 1 decision, 2 transport, 3 care) + facility action. Continuum breaks = dropout between ANC → birth → PNC.

★ PNC PEARL: Three delays explain most deaths — decision, transport, care.

► MDSR in one line?

Confidential no-blame audit of every maternal death to fix avoidable factors.

RESPECTFUL MATERNITY CARE (RMC)

RMC = dignity + privacy + consent + companion + ZERO abuse for EVERY woman! Disrespect is a rights violation — calm words + pain relief + choice of position change outcomes.

Dignity + privacy always	Companion of her choice	Zero slap/shout + pain relief
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★ RIGHT — BEDSIDE ACT

Right	Bedside act
Dignity + privacy	Screens, drape, whisper exams, cover
Consent + choice	Explain, ask, birth position + mobility
Companion	One chosen support person allowed
Equity	Same care regardless caste, pay, age

● Offer fluids + upright positions + intermittent monitoring in normal labour; avoid routine lithotomy + shouting.

● Report abuse, display charter + helpline, take feedback after birth.

⚠ FATAL TRAP

Slapping thighs, shouting 'push!' or denying pain relief to 'teach a lesson' is obstetric violence — complaint + trauma + avoids next facility birth! Coach breathing, praise effort, relieve pain.

★ NORCET HIT

HIT: RMC markers = greeted by name, consented exams, companion present, position of choice. LaQshya/Dakshata push RMC in labour rooms.

★ PNC PEARL: A frightened mother tenses — kindness literally shortens labour.

► Woman screams in labour; relative asks you to slap her awake. Act?

Refuse + calm coaching + pain relief + allow companion; document + counsel.

MLCU = midwife-led unit for LOW-RISK women — minimal intervention, mobility, intermittent auscultation, fixed referral thresholds + ready transport!

MLCU = low-risk ONLY	Midwife leads + refers	Transport always ready
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▲ FEATURE — MLCU — ROUTINE WARD

Feature	MLCU	Routine ward
Lead	Midwife	Doctor-led team
Eligibility	Low-risk screened	All risk levels
Monitoring	Intermittent IA + partograph	Often continuous CTG
Referral	Fixed thresholds + vehicle	In-house escalation

⚠ FATAL TRAP

Keeping a high-risk woman (HTN, bleeding, malpresentation, twins) in MLCU to 'avoid C-section' delays rescue! Re-screen EVERY visit; transfer at FIRST flag — thresholds save lives.

★ NORCET HIT

HIT: Physiologic birth supports = mobility + fluids + companion + upright pushing + delayed cord + skin-to-skin. IA every 15 min active labour.

★ PNC PEARL: MLCU success = strict entry + strict exit rules.

► Two instant MLCU referral triggers?

HTN/preeclampsia signs + any bleeding or malpresentation/non-reassuring FHR.

SRHR = autonomy + choice + confidentiality for ALL ages! ICM defines what EVERY midwife must do — pre-pregnancy → birth → postnatal, accountable + collaborative.

**Choice + privacy
every age**

**RKSK = youth
friendly**

**ICM = 4-domain
competence**

◆ DOMAIN — POINT

Domain	Point
Autonomy	Contraception + MTP counsel, voluntary, private
Youth (RKSK)	AFHS clinics, no judgment, counsel + condoms
GBV care	LIVES: listen, inquire, validate, safety, support + refer
ICM domains	Pre-pregnancy, pregnancy, labour-birth, postnatal-newborn

⚠ FATAL TRAP

Scolding an adolescent seeking contraception or informing parents without consent drives her to unsafe abortion! Private counsel, condoms + method, STI screen, follow-up date.

★ NORCET HIT

HIT: GBV first-line = LIVES + document injuries + MLC if assault + shelter/helpline 181/112. Accountability = records + incident + handover.

★ PNC PEARL: Confidentiality has ONE break-glass: imminent serious harm.

► 16-year-old asks contraception secretly. First 3 acts?

Private room + non-judgmental counsel + method/condoms + STI screen.

LEGAL: MTP, PC-PNDT, ADOPTION, SURROGACY

LAW protects mother + girl child! MTP per current rules + consent + records; PC-PNDT bans ANY sex disclosure; adoption ONLY via CARA; surrogacy altruistic + registered!

MTP = consent + records	PC-PNDT: NEVER reveal sex	CARA / registered only
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⚠ LAW — RULE — NURSE

Law	Rule	Nurse
MTP	Gestation + opinion rules per amendment	Consent + docs, no coercion
PC-PNDT	No sex determination/communication	Refuse + display ban + report pressure
Adoption	CARA legal process only	Never facilitate private handover
Surrogacy	Altruistic + registered clinic	Counsel legality + consent

⚠ FATAL TRAP

Whispering fetal sex 'as a joke' or writing it anywhere = jail + licence loss + clinic seal! Refuse every request, explain the ban, report coercion to MO + authority.

★ NORCET HIT

HIT: MTP needs written consent (guardian if minor) + registered place + opinion(s) per gestation + records 5 yrs. 181/112 for GBV/pressure cases.

★ PNC PEARL: Paper protects — consent + opinion + registers, every time.

► Family offers cash for fetal sex on USG. Response?

Refuse + explain PC-PNDT ban + document + inform MO; never comply.

REPRODUCTIVE ORGANS — OBSTETRIC REVIEW

KNOW the landmarks labour depends on! Fundus dates pregnancy, cervix dilates, **AMPULLA** fertilizes, Mackenrodt holds the cervix — lax = prolapse!

**Fundus = dates
pregnancy**

**Ampulla =
fertilization**

**Mackenrodt =
cervix support**

▲ ORGAN — KEY FACT — WHY IT MATTERS

Organ	Key fact	Why it matters
Uterus	Pear ~7.5 cm; fundus/body/cervix	Fundal height = gestation + growth
Cervix	Plug + effacement/dilatation	Bishop + labour progress
Tube	Ampulla fertilizes; isthmus tied	Ectopic #1 = ampulla
Ovary	Day-14 ovulation; corpus luteum	Luteal progesterone saves pregnancy
Mackenrodt	Transverse cervical ligament	Supports uterus; lax → prolapse

⚠ FATAL TRAP

Missing a bulky bleeding uterus with clots as 'normal PNC discharge' hides atony/retained bits! Feel fundus (firm + midline?), measure blood, inspect placenta + membranes fully.

★ NORCET HIT

HIT: Fertilization → ampulla; sterilization → isthmus; uterus artery crosses ureter ('water under bridge') — surgical caution.

★ PNC PEARL: Ampulla makes, isthmus blocks — one tube, two exam answers.

► Fertilization site + commonest ectopic site?

Ampulla of tube — both answers, same place.

STATION is measured FROM the ischial spines! Inlet OC ~10.5, MID IS-10 (smallest), outlet IT-11. Engagement = presenting part at 0 station!

OC ~10.5 inlet	IS-10 = smallest midplane	0 station = engaged
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Σ PLANE — LANDMARK — SIZE

Plane	Landmark	Size / clue
Inlet	Promontory → pubis	OC ~10.5; transverse ~13
Mid (cavity)	Ischial spines	IS 10 cm = SMALLEST
Outlet	Tuberosities	IT 11 cm
Joints	Sacroiliac + symphysis	Relaxin softens; Waddling gait

⚠ FATAL TRAP

Calling +2 station 'high and unengaged' misreads labour and delays decisions! Minus = above spines; 0 = engaged; plus = descending — station + dilatation + contractions = progress.

★ NORCET HIT

HIT: Pelvic inclination ~55°. serial exams need consent + privacy + handwash. Deep transverse arrest lodges at MID (IS-10).

★ PNC PEARL: Spines are the zero line of labour — everything stations from there.

► VE: presenting part at spines, cervix 8 cm. Station + meaning?

0 station = engaged; active-phase progress, continue IA + partograph.

FETOPELVIC TERMS: LIE TO ENGAGEMENT

Six words map EVERY labour! Lie → presentation → attitude → position (denominator!) → station → engagement. Denominator names the position — occiput, sacrum, mentum!

Denominator fixes position	Station -3...+3 from spines	0 = engaged at inlet
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◆ TERM — MEANS — EXAMPLE

Term	Means	Example
Lie	Fetal vs maternal axis	Longitudinal / transverse
Presentation	Part entering pelvis	Vertex, breech, shoulder
Attitude	Flexion degree	Full flexion = vertex
Position	Denominator vs pelvis	ROA, LOP, RSP
Station	Spines level	-3 ... 0 ... +3
Engagement	Widest part at inlet	At 0 station

⚠ FATAL TRAP

Documenting 'cephalic' alone for a transverse lie with prolapsed cord misses the emergency! Confirm lie + presentation + FHR together — cord prolapse = knee-chest + lift + O2 + OT!

★ NORCET HIT

HIT: ROA = occiput-right-anterior. Sacrum = breech denominator; mentum = face. Attitude decides diameter: flexed vertex SOB 9.5.

★ PNC PEARL: Name the denominator, and the position writes itself.

► ROA + fully flexed. Decode?

Vertex, occiput to maternal right-anterior — ideal for vaginal birth.

CYCLE, OVULATION, FERTILIZATION, IMPLANTATION

Day-14 LH surge releases the egg; ampulla fertilizes within 24h; blastocyst implants Day 6–10 and hCG rescues the corpus luteum — the whole story behind every pregnancy test!

LH surge = Day 14	Fertilize <24h in ampulla	Implant D6–10 + hCG
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▲ PHASE / EVENT — KEY — HORMONE

Phase / Event	Key	Hormone
Proliferative	Days 5–14	Estrogen rebuilds lining
Ovulation	~Day 14, mittelschmerz	LH surge
Secretory	Days 15–28	Progesterone matures
Fertilization	Ampulla, <24 h	Zygote → morula → blastocyst
Implantation	Day 6–10, upper-posterior	hCG → test positive

⚠ FATAL TRAP

Dating pregnancy from quickening or fundal guess instead of LMP + early USG misplaces EDD and mistimes steroids/induction! Naegele (LMP –9 mo +7 d) + first-trimester USG = gold.

★ NORCET HIT

HIT: Ovulation kits track LH. Luteal phase fixed ~14 d; follicular varies. hCG doubles ~48 h in viable early pregnancy.

★ PNC PEARL: Luteal is clockwork (14 d) — follicular sets the cycle length.

► LMP 10 Jan. EDD by Naegele?

17 October (Jan –3 = Oct, 10+7 = 17).

Three layers build every organ; placenta feeds + breathes + hormones! After birth INSPECT both faces + membranes + cord vessels — retained bits bleed!

Ecto/meso/endo 3 layers	Shiny = fetal Rough = maternal	Count lobes + vessels!
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Σ ITEM — MILESTONE / ROLE

Item	Milestone / role
Germ layers	Ecto (skin/brain), meso (heart/muscle), endo (gut/lungs)
Viability	~24 wks current concept — NICU threshold
Maternal face	Rough cotyledons 15–20 — count them
Fetal face	Shiny Schultze + vessels + membranes
Hormones	hCG + progesterone + estrogen + hPL

⚠ FATAL TRAP

Tossing the placenta without inspection hides retained cotyledon/succenturiate lobe — hours later: boggy uterus + PPH! Spread membranes, count lobes + vessels (2A+1V), save for review.

★ NORCET HIT

HIT: Single umbilical artery → anomaly screen.
Battledore/velamentous cord → vasa-previa bleed risk.
hPL readies lactation.

★ PNC PEARL: Rough-reassure, shiny-sigh — but COUNT before you relax.

► Placenta shows 2 vessels in cord. Meaning + next?

Single umbilical artery — anomaly scan + neonatal watch.

FETAL CIRCULATION + THREE SHUNTS

Fetal lungs sleep, so **THREE** shunts bypass them! Remember 1-2-3: 1 vein IN (oxygen), 2 arteries OUT (waste), 3 shunts (venosus, ovale, arteriosus)!

1 vein IN oxygen	2 arteries OUT waste	3 shunts skip liver + lungs
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★ SHUNT — BYPASSES — BECOMES

Shunt	Bypasses	Becomes
Ductus venosus	Liver → IVC	Ligamentum venosum
Foramen ovale	Right → left atrium	Fossa ovalis
Ductus arteriosus	Pulmonary → aorta	Ligamentum arteriosum
Umbilical vein (1)	Carries oxygen IN	Ligamentum teres
Umbilical arteries (2)	Carry waste OUT	Medial ligaments

⚠ FATAL TRAP

Clamping a preterm cord instantly while baby is flat + cold shocks transition! Delayed cord (1–3 min) + warmth + position at/below placenta — unless resuscitation demands cut.

★ NORCET HIT

HIT: At birth lungs expand → shunts close (arteriosus: days; ovale: hours–days). Persistent PDA murmur + bounding pulses = echo + fluids/NSAIDs per order.

★ PNC PEARL: 1-2-3: vein-arteries-shunts — count them on every diagram.

► Right-to-left atrial shunt in fetus?

Foramen ovale — closes to fossa ovalis after birth.

PRECONCEPTION CARE

Pregnancy health is decided BEFORE conception! Folate 400 mcg × 3 months prior + disease control + vaccines + spacing — neural tube closes by Day 28, before she knows!

Folate 400mcg ×3 mo prior	Control DM/HTN stop teratogens	Space 2–3 yrs + vaccines
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★ CHECK — DO

Check	Do
Nutrition/folate	400 mcg daily 3 mo pre + fix anemia
Disease + drugs	Control DM/HTN/thyroid; stop valproate/isotretinoin
Infections + vax	Rubella immune, Td current, treat STI/UTI
Lifestyle	No tobacco/alcohol, healthy BMI, 2–3 yr spacing

⚠ FATAL TRAP

Starting folate AFTER the missed period misses neural-tube closure (Day 28)! Counsel EVERY newlywed/adolescent bride — folate + ANC booking + spacing talk at FIRST contact.

★ NORCET HIT

HIT: Preconception package = folate + Hb + BP + sugar + thyroid + vax + spacing + tobacco screen. NTD risk highest with valproate + DM.

★ PNC PEARL: 400 mcg × 3 months BEFORE — the cheapest brain insurance.

► Newly married, planning in 6 months. One drug + duration?

Folic acid 400 mcg daily for 3 months before conception.

Only THREE positives prove pregnancy: fetal heart + ultrasound fetus + examiner-felt movements! UPT and Hegar only suggest — confirm before ANC labelling!

Presumptive = her story	Probable = exam + UPT	POSITIVE = heart scan + felt moves
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▲ SIGN — EXAMPLE

Sign	Example
Presumptive	Amenorrhea, nausea, Chadwick/Goodell, quickening
Probable	Hegar + ballottement + Braxton Hicks + UPT positive
Positive	FHS by Doppler, USG fetus, palpable fetal movements
⚠ FATAL TRAP Declaring pregnancy on UPT alone and starting 'ANC' for a molar/ectopic misses the emergency! Confirm with USG (sac + cardiac activity) + dates; bleed/pain → ectopic/mole workup FIRST.	★ NORCET HIT HIT: hCG doubles ~48 h viable; discriminatory zone ~gest sac at 1500–2000 (TVS). Hegar = soft isthmus 6–8 wks; Goodell = soft cervix.

★ PNC PEARL: Heart, scan, felt moves — the only three that PROVE.

► UPT positive + vaginal bleed + adnexal pain. First fear + test?

Ectopic — vitals + USG + quantitative hCG + resuscitation readiness.

PHYSIOLOGICAL CHANGES IN PREGNANCY

Pregnancy rewrites physiology: volume +50% (dilutional anemia!), clotting UP, GFR UP, gut SLOW, spine CURVES. Know normal — or every adaptation looks like disease!

Volume +50% Hb falls	Clotting UP = DVT risk	GFR up, gut slow skin marks
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Σ SYSTEM — CHANGE — EFFECT

System	Change	Effect
Heart	CO +40%, SVR down	M murmur ok; supine IVC press
Blood	Plasma > RBC rise	Hb 11 cutoff; hypercoagulable
Lungs	Tidal volume up	Dyspnea normal; PaCO2 low
Kidney	GFR up +50%	Glycosuria ok; UTI frequent
GI + skin	Slow gut; stretch marks	Heartburn, piles; linea/striae

⚠ FATAL TRAP

Treating ankle edema + breathlessness at 34 wks as 'heart failure' — or IGNORING sudden face swelling + headache as 'normal' — both kill! Gradual = adapt; SUDDEN + HTN/protein = preeclampsia.

★ NORCET HIT

HIT: Supine hypotension → left lateral + wedge. Lordosis + relaxin = backache + waddle. Chloasma + linea nigra fade postpartum.

★ PNC PEARL: Gradual whispers adaptation; sudden shouts danger.

► 36 wks, dizzy supine, fine on side. Cause + fix?

IVC compression — left lateral tilt + wedge, vitals recheck.

ANC HISTORY, EDD & GRAVIDA-PARA

LMP + early USG date the pregnancy; G-P-A-L tells the story! Naegele (LMP –3 mo +7 d) + <14-week scan = gold — dates decide steroids, induction, everything!

**Naegele: –3mo
+7 days**

**<14w USG beats
memory**

**G-P-A-L tells
the story**

◆ ITEM — ASK / RECORD

Item	Ask / record
LMP + EDD	Naegele; confirm <14 wks USG
G-P-A-L	Gravida, preterm/term, abortus, living
History	Medical, surgical, family, tobacco, GBV
Exam	Vitals + edema + pallor + breasts + pelvis

⚠ FATAL TRAP

Dating by quickening/fundal guess misplaces EDD — steroids too early, induction too late! Always LMP + early USG; unsure dates → senior + serial scans, never guess.

★ NORCET HIT

HIT: 8 ANC contacts minimum. First visit <12 wks: full labs + Td + IFA + counsel. High-risk tag (age, HTN, DM, twins, prior CS) at visit ONE.

★ PNC PEARL: Under-14-week scan beats every memory.

► LMP 5 May. EDD?

12 February (May – 3 = Feb, 5 + 7 = 12).

LEOPOLD MANEUVERS + FUNDAL HEIGHT

Hands map the fetus: fundus → lateral → Pawlik → pelvic! McDonald tape cm ≈ weeks (20–36) — small/large for dates = growth scan NOW!

Fundal→lateral→ Pawlik→pelvic	McDonald cm ≈ weeks	Empty bladder first!
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▲ GRIP — FINDS

Grip	Finds
Fundal	Breech (soft) vs head (hard round) on top
Lateral	Back (smooth) vs limbs (knobby)
Pawlik	Presenting part + ballotable or fixed
Pelvic	Engagement + attitude + descent

⚠ FATAL TRAP

Palpating over a FULL bladder or pushing hard on a contracted uterus fakes lie + hurts! Empty bladder, warm hands, wait between contractions, consent + drape always.

★ NORCET HIT

HIT: 20 wks = umbilicus; 36 wks = xiphisternum. Lag >2 cm → IUGR/oligo scan; excess → twins/poly/GDM workup.

★ PNC PEARL: Tape lies only if bladder is full — empty it first.

► Fundus 2 cm below umbilicus. Weeks?

~18 weeks (umbilicus = 20).

FETAL HEART ASSESSMENT

110–160 + heard on the BACK side! Vertex = below umbilicus, breech = above. Any <110 / >160 / decels = left lateral + O2 + senior — never ‘wait and watch’ alone!

110–160 normal	Back side = loudest	Vertex below breech above
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★ TOOL — NOTE

Tool	Note
Pinard	18–20 wks+, needs skill + quiet room
Doppler	12 wks+, count 1 full minute
Site	Back side; vertex below umbilicus
Abnormal	<110/>160, decels → position + O2 + call

⚠ FATAL TRAP

Counting for 15 seconds × 4 during a contraction hides decelerations! Count a FULL minute BETWEEN contractions + feel maternal pulse together — doubling = mother, not baby!

★ NORCET HIT

HIT: Pinard angle toward fetal back. Meconium + abnormal FHR = continuous monitoring + OT readiness. Document rate + rhythm + action.

★ PNC PEARL: Back-side rule locates presentation without a scan.

► Loudest FHR above umbilicus right. Likely presentation?

Breech (buttocks up) — confirm by Leopold + USG.

Labs catch the silent killers: Hb (anemia), Rh (HDN), urine protein (preeclampsia), sugar (GDM), HIV/syphilis/HBsAg! Repeat Hb + urine EVERY visit!

Hb + urine EVERY visit	Rh-neg → ICT + anti-D	GDM 24–28w OGCT
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Σ TEST — WHEN / WHY

Test	When / why
Hemoglobin	Each visit; <11 anemia, <7 severe
Group + Rh	Once; Rh-neg → ICT + anti-D 28 wks
Urine	Protein (PE) + sugar + UTI each visit
GDM screen	24–28 wks OGCT; early if risk
HIV/syphilis/HBsAg	Once + treat; partner test

⚠ FATAL TRAP

Skipping ICT in Rh-negative or ignoring trace protein with rising BP misses HDN/preeclampsia until seizure! Protocol ranges + action thresholds on the card — escalate, never file-and-forget.

★ NORCET HIT

HIT: Syphilis RPR+ → benzathine penicillin SAME visit. Bacteriuria treat even if silent (preterm trigger). Hb electrophoresis if microcytic + family history.

★ PNC PEARL: Protein + pressure = preeclampsia workup NOW.

► Rh-neg, 28 wks, ICT negative. Give?

Anti-D 300 mcg + repeat ICT + cord-blood plan at birth.

Same care, two traps: treat the nuisance, NEVER miss its dangerous twin! Nausea vs hyperemesis, edema vs preeclampsia, discharge vs ROM — sudden + severe = red flag!

Small-dry-frequent meals win	Sudden + severe = red flag	Hydrate + move + posture
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◆ COMPLAINT — RELIEF — RED FLAG

Complaint	Relief	Red flag
Nausea	Dry toast, ginger, B6	Ketones + weight loss
Heartburn	Head-up, small meals	Severe epigastric + HTN
Constipation/piles	Fiber + fluids + walk	Bleeding heavy
Cramps/edema	Calcium, elevation	Face swelling, headache
Discharge/frequency	Hygiene, timed voids	Gush (ROM) / dysuria

⚠ FATAL TRAP

Sending home vomiting + ketones + 5% weight loss with 'eat ginger' starves mother + fetus! Hyperemesis = fluids + thiamine + electrolytes + admission per severity.

★ NORCET HIT

HIT: Carpal, round-ligament, backache → posture + belt + physio. Itching palms/soles + dark urine → cholestasis bile-acid workup!

★ PNC PEARL: Minor today, major if sudden — re-examine every repeat visit.

► Itchy palms/soles + dark urine at 32 wks. Suspect + test?

Cholestasis — bile acids + LFT + fetal surveillance + delivery plan.

ANC NUTRITION, IFA, CALCIUM & WEIGHT

+350 kcal + 78 g protein + IFA 100+ days + calcium 1 g + 10–12 kg gain! Vit C WITH iron, calcium APART, NO tea with tablets — absorption is everything!

+350 kcal 78 g protein	IFA + Vit C no tea	Calcium apart 10–12 kg gain
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▲ NEED — COUNSEL

Need	Counsel
Calories/protein	+350 kcal; milk, dal, egg, groundnut
IFA	100+ days; Vit C; tea/coffee gap 2 h
Calcium + D	1 g (apart from IFA) + sunlight
Weight	10–12 kg; track each visit
Safety	Wash + cook + iodized salt + deworm

⚠ FATAL TRAP

IFA with morning tea + no recheck lets Hb slide to transfusion level by term! Watch black stools (expected), fix constipation, recheck Hb in 4 wks — non-rise = compliance/parasite/malaria workup.

★ NORCET HIT

HIT: Severe anemia (<7) → transfusion plan BEFORE labour (never in active labour if avoidable). Twins + teens + short spacing = higher dose vigilance.

★ PNC PEARL: C with iron, calcium alone, tea never.

► Hb 9.5 at 30 wks, tea with tablets. Plan?

Therapeutic IFA + Vit C timing + deworm + diet + Hb recheck 4 wks.

IMMUNIZATION IN PREGNANCY (Td)

Td in EVERY pregnancy per current schedule + clean cord + skilled birth = neonatal tetanus ZERO! Live vaccines WAIT till postpartum — counsel the gap, never skip Td!

Td EVERY pregnancy	Unimmunized: 0-4w-6mo	Live vax AFTER birth
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★ STATUS — SCHEDULE

Status	Schedule
Never immunized	Td1 now + Td2 4 wks + Td3 6 mo later
2 doses, last <3 yrs	Single booster this pregnancy
Unknown card	Treat as unimmunized + document
Postpartum	Catch-up + rubella/MMR if needed (avoid conception 1 mo)

⚠ FATAL TRAP

Giving MMR/varicella IN pregnancy or skipping Td 'for next visit' risks rubella syndrome / tetanus! Screen card at visit ONE; Td same-day; live vax booked postpartum.

★ NORCET HIT

HIT: Td protects BOTH (IgG crosses). Six cleans: hands, surface, blade, cord tie, stump, no application. TT-neonatal link is a favourite one-liner.

★ PNC PEARL: Card at visit one, Td same day — never 'next time'.

► 20 wks, zero prior doses. Dates?

Td1 today + Td2 at 24 wks + Td3 six months later.

True labour = regular + rhythmic + STRONGER + show/ROM + cervical change! Braxton Hicks fades with walk/rest. 5-1-1 + show/ROM = come NOW — phone-triage every caller!

Regular+rhythmic + stronger	Show + ROM = come now	Cervix decides admit or walk
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◆ SIGN — TRUE — FALSE / ACT

Sign	True labour	False / act
Contractions	Regular, longer, stronger	Irregular → walk/rest/fluids
Show	Blood-mucus plug	Wipe + watch
Membranes	Pooling + ferning	Leak query → sterile speculum
Decision	Tightening + change	Time 1 h + call; Gush/cord/bleed = RUSH

⚠ FATAL TRAP

Admitting every Braxton Hicks clogs labour rooms — but sending home true ROM/bleed/SRFM infects/kills! Triage by phone + card: fever, waters, bleed, less moves, HTN = come NOW.

★ NORCET HIT

HIT: Pre-labour ROM at term + Group-B unknown → antibiotics per protocol + induction window. Cord prolapse (gush + something per vaginum) = knee-chest + lift + OT!

★ PNC PEARL: 5-1-1 + show/ROM — the admission password.

► Gush of fluid + cord felt at vulva. First 3 moves?

Knee-chest + gloved hand lift + O2 + emergency OT call.

STAGES OF LABOUR

Four stages, two gates: 6 cm (active), FULL + urge (second)! Latent 0–6 slow, active 6–10 fast + descent, second = push + FHR, third = placenta <30 min, fourth = 2-h bleed watch!

6 cm = active gate	Full + urge = push	Placenta <30 min + 2-h watch
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Σ STAGE – EVENTS – CLUE

Stage	Events	Clue
1st latent	0–6 cm, effacement	Slow; walk + fluids
1st active	6–10 cm + descent	Fast; partograph + IA
2nd	Full → birth	Push with urge; FHR q5–15
3rd	Placenta separates	AMTSL; >30 min = retained
4th	1–2 h recovery	Fundus + vitals + bleed q15

⚠ FATAL TRAP

Pushing at 8 cm 'to hurry' swells the cervix and tears it! Confirm FULL + station with VE before pushing; occiput-posterior + deflexion need patience + position change, not force.

★ NORCET HIT

HIT: Friedman vs Zhang: active from 6 cm modern. Prolonged: nullipara >20 h / multipara >14 h flags. Second stage: primi <3 h (epi), multi <2 h.

★ PNC PEARL: Six opens the gate, ten opens the door.

► 8 cm, urge to push, VE rim felt. Allow?

No — anterior lip; side-lying + pant/blow + recheck, no forced pushing.

LABOUR-ROOM TRIAGE (10-MINUTE RULE)

Ten minutes decide the room: vitals + protein, FHR + lie/ROM, risk + card + blood group, then admit/labour/OT + consent + companion! Eclampsia/bleed/cord jump the queue!

Vitals+protein first	FHR+lie+ROM together	Card+blood group + consent
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★ STEP — 10-MIN DO

Step	10-min do
Mother	BP + protein, temp, pulse, pallor, edema
Fetus	FHR + lie/presentation + ROM/meconium
History	Parity, risk, ANC card, blood group, allergies
Plan	Admit/labour/OT + consent + companion + NPO plan

⚠ FATAL TRAP

Repeated VE without indication/consent seeds chorioamnionitis! VE only for: admission staging, progress doubt, ROM check, urge before full — sterile + minimal + document
Bishop/dilatation/effacement/station/membranes.

★ NORCET HIT

HIT: Red jumpers = cord prolapse, APH, eclampsia, shoulder dystocia drill, PPH kit. SBAR handover + partograph started at active phase.

★ PNC PEARL: Triage sorts, partograph proves.

► Triage: BP 150/100 + protein 2+ + headache. Next?

Preeclampsia protocol + MgSO4 workup + labs + senior + delivery plan.

VE only with indication + consent + sterile hands! Document 5 things: dilatation, effacement, station, position, membranes — minimal exams prevent chorioamnionitis!

Indication only	Sterile + consent + drape	5 Ds document
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◆ WHEN — HOW / RECORD

When	How / record
Admission staging	Consent, bladder empty, Bishop score
Progress doubt	4-hourly in active; dilatation + descent
ROM query	Sterile speculum first: pooling/ferning
Urge before full	Confirm rim vs full + station

⚠ FATAL TRAP

Hourly routine VE 'to show progress' seeds chorioamnionitis, especially after ROM! Indication + sterile + few + record — each VE must answer a question.

★ NORCET HIT

HIT: Bishop = dilatation + effacement + station + consistency + position. Never VE with APH undiagnosed placenta previa — speculum + USG first!

★ PNC PEARL: Every VE answers a question — else hands off.

► VE: 7 cm, 80%, -1, intact. Read?

Active phase, high station — progress with contractions, IA on.

PARTOGRAPH / WHO LABOUR CARE GUIDE

Plot from ACTIVE (6 cm)! Dilatation + descent + contractions + FHR + maternal vitals — crossing the ACTION line means decide: ARM, oxytocin review, or CS call!

Start at 6 cm active	Alert → action 4 hours	Plot FHR + Cx + descent
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Σ PLOT — ACTION

Plot	Action
Dilatation curve	Alert then action line; decide on cross
Descent + station	Marks falling = progress
Contractions/10 min	Shade boxes; adequate = 3–5
FHR + maternal	Below graph; BP/temp/pulse + urine

⚠ FATAL TRAP

Starting the partograph in LATENT phase or leaving the action column blank after crossing guarantees missed arrest! Start at 6 cm, plot EVERY 30 min–hourly, act + document on cross.

★ NORCET HIT

HIT: LCG adds companion + pain + position + fluids + respectful care sections. Arrest: no change 4 h adequate Cx (6 cm+) = senior + CPD review.

★ PNC PEARL: Crossed line, made decision — always paired.

► 7 cm, 4 h, no change, adequate contractions. Call?

Active-phase arrest — senior review for CPD/OT decision.

IA: 30-min latent, 15-min active, 5-min second stage! Variability = happy brain; lates + meconium-thick = position + O2 + oxytocin OFF + senior + OT ready!

IA 30-15-5 rule	Variability = happy brain	Thick mec + late = OT-ready
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★ METHOD — RULE

Method	Rule
IA Doppler/Pinard	30' latent, 15' active, 5' second + post-Cx
CTG	High-risk continuous; baseline 110–160
Meconium	Thin watch; thick + abnormal = expedite
Rescue	Left + O2 + fluids + stop oxytocin + call

⚠ FATAL TRAP

Flat supine CTG + pressure crash while 'watching variability' starves the fetus! Position lateral + wedge FIRST, then read — hypotension mimics distress and treatment differs.

★ NORCET HIT

HIT: Early = head compression (mirror Cx, benign); late = uteroplacental failure; variable = cord. Tachycardia + fever = chorioamnionitis screen!

★ PNC PEARL: Variability reassures more than any single number.

► Late decels + thick meconium + 8 cm. Bundle?

Lateral + O2 + oxytocin off + senior + OT prep + document.

PAIN RELIEF + MOBILITY + COMPANION

Choice + companion + upright + fluids = faster, kinder labour! Drugs only on request with monitoring — pethidine near birth sedates the BABY too!

Companion halves
distress

Upright + walk
= faster

Drugs on request
+ monitor

▲ METHOD — NOTE

Method	Note
Breathing/massage/water	First-line; companion coaches
Upright + walk	Shorter labour, less pain
Entonox/pethidine	Request-based; pethidine far from birth
Epidural	Consent; BP + bladder + FHR watch

⚠ FATAL TRAP

Pethidine 1 hour before birth + no resuscitation ready = floppy non-breathing baby! Avoid near second stage; naloxone + PPV corner ready if given — document time + dose.

★ NORCET HIT

HIT: Epidural needs preload + left tilt + bladder care + top-up plan. Refusing ALL relief while suffering = offer again kindly, RMC always.

★ PNC PEARL: Move, sip, companion — before any needle.

► Epidural + BP 88/58 + FHR dip. Bundle?

Lateral + fluids + O2 + stop top-up + call anaesthetist.

MECHANISM OF LABOUR (VERTEX LOA)

Seven moves, one mantra: flexion shrinks, rotation finds space, extension is born! Engagement → descent + flexion → internal rotation → extension → restitution → external rotation → expulsion!

Flexion = small
SOB 9.5

Rotation to
pubis

Restitution then
shoulders

◆ STEP — MOVE

Step	Move
Engagement	SOB biparietal at inlet
Descent + flexion	Chin to chest; SOF presents
Internal rotation	Occiput rotates to symphysis
Extension	Crowning; Ritgen guards
Restitution + ext. rotation	Head turns; shoulders align
Expulsion	Anterior shoulder first, then body

⚠ FATAL TRAP

Pulling the head before restitution or yanking shoulders tears plexus + perineum! Hands-off till restitution, check neck cord loosely, deliver anterior shoulder first with gentle guidance.

★ NORCET HIT

HIT: Asynclitism + deflexion jam rotation — position change + patience. OP needs long rotation; face-to-pubis delivers by flexion!

★ PNC PEARL: Flexion shrinks, rotation seeks, extension greets.

► Head out, face to mother's thigh. Next?

Wait restitution — feel cord, deliver anterior shoulder first.

CONDUCT OF NORMAL VAGINAL BIRTH

Five cleans + warm corner + guarded hands + ZERO fundal push! Head by flexion, shoulders anterior-first, cord 1–3 min, oxytocin 10 IU — calm room, ready resuscitation!

5 cleans ready	Guard, don't pull	Cord 1–3 min + oxytocin
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Σ STEP – DO

Step	Do
Prep	5 cleans + warm towels + PPV corner + team
Head	Maintain flexion, Ritgen, suction only if needed
Shoulders/body	Anterior first, gentle, support
Cord + drug	Clamp 1–3 min; oxytocin 10 IU IM
Placenta	CCT only with signs + contraction

⚠ FATAL TRAP

Fundal pressure ('helping push') ruptures uterus + injures baby + banned! Upright + urge + patience; outlet forceps/vacuum only by trained hands with indication + consent.

★ NORCET HIT

HIT: Hands-on vs poised debated — perineal support + warm compress protect. Delayed cord boosts Hb/iron; milking only if early cut needed.

★ PNC PEARL: Calm hands, guarded head, timed drug.

► **Gush + fundus rises + cord lengthens. Means + move?**

Separation — CCT with counter-pressure + inspect placenta.

IMMEDIATE NEWBORN CARE (GOLDEN MINUTE)

Warm – dry – stimulate – position – PPV if apneic! Then skin-to-skin + latch <1 h + Vit K + weigh + band. Bath waits 24 h — cold kills quietly!

Warm-dry-stim 60 seconds	Skin-to-skin + latch <1h	Vit K + weigh + ID band
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★ STEP — DO

Step	Do
Warm chain	Dry + cap + warm surface, no bath
Breathe	Position + clear only if blocked; PPV if apneic
Cord	Clamp 1–3 min; check 2A + 1V
Bond + feed	Skin-to-skin + latch within hour
Protect	Vit K + eye care + weigh + band

⚠ FATAL TRAP

Bathing + vigorous foot-slapping a quiet baby wastes the golden minute! Dry-stimulate 30 s, position airway, PPV if no cry — suction ONLY visible obstruction, routine deep suction harms.

★ NORCET HIT

HIT: Apgar 1 + 5 min guides resuscitation depth. Single artery → anomaly watch. Footprint + band + photo prevent swaps!

★ PNC PEARL: Golden hour: warm + skin + suck.

► Term, limp, apneic after dry-stim. First 3?

Position airway + PPV + call NRP team + clock running.

Retroplacental clot + contraction shear the placenta in <30 min! Schultze (shiny, 80%) vs Duncan (rough edge) — gush + rise + lengthen = separated, deliver NOW with guard!

Gush+rise+length = separated	Schultze shiny 80%	<30 min or retained!
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▲ SIGN — MEANS

Sign	Means
Gush + fundal rise	Separated — deliver
Cord lengthens	Descent into vagina
Schultze	Fetal shiny side first
Duncan	Maternal rough edge first
Beyond 30 min/bleed	Retained — explore/OT

⚠ FATAL TRAP

Cord traction BEFORE separation + soft uterus inverts the uterus inside-out! Wait signs + contraction, counter-pressure ALWAYS, stop + massage if resistance — inversion = shock + replace + OT!

★ NORCET HIT

HIT: Brandt-Andrews = one hand suprapubic push + other cord hold. Placenta accreta risk climbs with prior CS + previa — never force!

★ PNC PEARL: Three S: signs, Schultze, swift under thirty.

► 35 min, no placenta, no bleed. Call?

Retained — OT/manual-removal prep + IV + blood ready.

AMTSL (ACTIVE MANAGEMENT)

Drug + traction + tone + tissue! Oxytocin 10 IU IM <1 min + CCT with counter-pressure + q15 massage + full inspection — AMTSL halves PPH!

Oxytocin 10 IU <1 minute	CCT + counter pressure	Massage + inspect placenta
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◆ COMPONENT — DETAIL

Component	Detail
Uterotonic	Oxytocin 10 IU IM within 1 min
CCT	With contraction + signs + counter-pressure
Tone	Massage; fundus firm + midline q15
Tissue	Placenta + membranes + vessels + perineum

⚠ FATAL TRAP

CCT on a SOFT uterus without counter-pressure inverts or snaps the cord! Confirm contraction + signs first; if cord snaps, hand in vagina + placenta support — never fish blindly with instruments untrained.

★ NORCET HIT

HIT: Cold chain for oxytocin/ergometrine.
Ergometrine/misoprostol where oxytocin absent or atony persists — plus TXA <3 h in PPH!

★ PNC PEARL: Drug-traction-tone-tissue — four Ts that stop floods.

► AMTSL done, steady atonic trickle. Next 3?

Massage + empty bladder + 2nd uterotonic + bimanual/condom-balloon prep.

Degrees climb inward: skin → muscle → sphincter → rectum! Episiotomy = restrictive mediolateral ONLY with indication + local + layered repair — never routine!

1-2-3-4 climbs inward	Restrictive epi mediolateral	Repair + ice + inspect
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Σ DEGREE — INVOLVES

Degree	Involves
1st	Skin/mucosa only
2nd	Muscle, sphincter intact
3rd (a/b/c)	Sphincter partial–full
4th	Rectal mucosa too
Episiotomy	Mediolateral 7–8 o'clock + local

⚠ FATAL TRAP

Routine episiotomy 'to save tears' + repair without anesthesia + missed rectal buttonhole = fistula + pain + lawsuit! Indication + consent + local + rectal check before AND after repair.

★ NORCET HIT

HIT: 3rd/4th = senior/doctor OT repair + antibiotics + stool softener + follow-up. Ice 24 h, sitz after, pad front-to-back, report pus/gaping.

★ PNC PEARL: Skin, muscle, sphincter, rectum — count inward, repair outward-safe.

► Repair done, rectal check shows stitch in mucosa. Act?

Remove + redo layer + recheck — never leave mucosal stitch.

Contraception BEFORE discharge + 2-hour bleed watch! CuT <48 h with consent + strings plan, then vitals q15 + fundus + pads + void <6 h — most PPH strikes HERE!

CuT <48 h + consent	Strings + 6-w check	2-h vitals + void <6 h
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★ ITEM — COUNSEL / DO

Item	Counsel / do
PPIUCD	CuT 380A <48 h, sterile, strings trimmed
Avoid	Fever, PPH, ROM >18 h, anomaly
Follow-up	6 wks: strings + USG if lost
4th stage	Vitals q15, fundus, pads, void <6 h

⚠ FATAL TRAP

Inserting PPIUCD with fever/foul discharge/PPH to 'finish fast' seeds PID + expulsion! Screen first; defer + bridge (condoms/DMPA) + dated return beats forced insertion.

★ NORCET HIT

HIT: Missing strings + pregnancy doubt → test + USG, NO blind pull. Void <6 h rules retention; fundus deviated = full bladder first!

★ PNC PEARL: Discharge with method + date = protected puerperium.

► 8 h, no void, dull suprapubic. Act?

Encourage + privacy + warm water; catheterize if fails + measure.

Unwritten = undone in court! Partograph complete + birth register (time, sex, weight, Apgar, placenta) + MCTS/RCH + SBAR handover — action column NEVER blank after crossing lines!

Partograph complete	Register + Apgar + placenta	MCTS + SBAR handover
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▲ RECORD — MUST INCLUDE

Record	Must include
Partograph	Plots + actions + decisions + outcome
Birth register	Time, sex, weight, Apgar, cord, placenta
MCTS/RCH + JSY	IDs + services + payments linkage
Handover	SBAR + risks + pending + plan

⚠ FATAL TRAP

Blank action column + 'verbal orders' after shoulder dystocia/PPH collapses in audit! Time-stamp maneuvers + drugs + team + baby status + debrief + incident where needed.

★ NORCET HIT

HIT: MLC births (assault/unknown/poison/brought-dead) = police + samples + chain intact. Debrief mother + companion before shifting!

★ PNC PEARL: Ink today beats memory tomorrow — in court and in care.

► Shoulder dystocia resolved in 3 min. Document?

Times + McRoberts/suprapubic/episiotomy + team + Apgar + cord gases + debrief.

PHYSIOLOGY OF NORMAL PUERPERIUM

Six weeks back to normal! Uterus shrinks 1 cm/day, lochia runs R-A-Y (red→pink→yellow-white), ovulation waits on breastfeeding — foul + fever + boggy = endometritis, ACT!

1 cm/day involution	Lochia R-A-Y 4–6 wks	EBF delays ovulation
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★ CHANGE — NORM

Change	Norm
Uterus	1 cm/day; pelvic by 2 wks, normal 6 wks
Lochia	Rubra → serosa → alba; ~4–6 wks
Ovulation	EBF delays; non-lactating 6–10 wks
Vitals	Temp <38 °C, pulse settles, BP normal

⚠ FATAL TRAP

Calling Day-4 foul lochia + 39 °C + tender boggy uterus 'milk fever' delays sepsis care! Endometritis = broad antibiotics + vitals + fluids + empty uterus check — admit, never home-antipyretic alone.

★ NORCET HIT

HIT: Subinvolution flags = boggy + heavy + clots + backache. Breast engorgement fever <24 h + well = milk; beyond = mastitis/infection hunt.

★ PNC PEARL: R-A-Y flows one way — backward (red again) means review.

► Day-3 boggy uterus + heavy clots. First 3?

Massage + empty bladder + retained-bits/bleed watch + vitals.

POSTNATAL ASSESSMENT (MOTHER + BABY)

Every check = dyad! Breasts, uterus-bladder-bowel, perineum, legs, mood + baby feed-stool-cord-jaundice. Legs + mood are the two most skipped — DVT + depression hide there!

Dyad every
check

Legs + mood
never skip

Baby alongside
always

▲ CHECK — LOOK FOR

Check	Look for
Breasts	Latch, cracks, engorgement, mastitis wedge
Uterus/bladder/bowel	Tone, void <6 h, flatus/stool
Perineum	REEDA: redness, edema, ecchymosis, discharge, approximation
Legs + mood	Calf pain/swelling; blues screen
Baby	Suck, stools, cord, jaundice zone

⚠ FATAL TRAP

Massaging a hot red calf 'for pain relief' embolizes a DVT to lungs! Unilateral swelling/pain = NO massage + Doppler + anticoag workup + mobilize rest of body only.

★ NORCET HIT

HIT: REEDA scores healing daily. Fundus deviated + atonic = bladder first. Home visits day 3 + 7 by ASHA catch sepsis/jaundice early.

★ PNC PEARL: Calf + cry: legs today, mood tomorrow — check both daily.

► Day-6 unilateral calf pain + swelling. Act?

Suspect DVT — no massage, Doppler, anticoag per order.

Pee in 6, poop with ease! Ice first 24 h → sitz later, pad front-to-back, void <6 h measured, fiber + fluids + walk — straining wrecks fresh repairs!

Ice 24 h → sitz later	Void <6 h measured	Fiber + walk no strain
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◆ CARE — DO

Care	Do
Perineum	Ice 24 h, sitz after, air-dry, front-to-back pads
Bladder	Void <6 h; retention = in-out/catheter + record
Bowel	Fiber + fluids + walk + softener; no strain
Wound	Inspect pain/gaping/pus; 3rd-degree = soft stool plan

⚠ FATAL TRAP

Straining on day-2 with 3rd-degree repair + constipation bursts the sphincter fix! Stool softener + fluids + footstool + 'breathe, don't push' — report incontinence/flatus leak SAME day.

★ NORCET HIT

HIT: Retention with overflow = atonic bladder; timed voids + double-void + privacy + warm water first. UTI dysuria + fever = culture + treat.

★ PNC PEARL: Six-hour bladder, soft first stool — the puerperium passports.

► Day-2 no stool + fear of pain. Plan?

Fiber + fluids + walk + softener + footstool + reassure + review.

MINOR PUERPERAL ISSUES + 6-WEEK FOLLOW-UP

Afterpains + engorgement + sore nipples bow to feeds + NSAIDs + latch fix — but fever, foul, flood, calf pain, or blues >2 wks = RED card, come NOW! Review at 6 weeks with baby!

Feed through fullness	Red card: fever/foul/flood	6-w review + baby
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Σ ISSUE / VISIT — ACT

Issue / visit	Act
Afterpains	NSAID + void; EBF cramps normal, fade days
Engorgement	Frequent feeds + warm pre/cold post + latch fix
Home visits	Day 3 + 7 ASHA: feed, cord, jaundice, mood
6-week review	Mother + baby + Hb + FP + immunization

⚠ FATAL TRAP

Stopping feeds for engorgement + binding breasts spikes mastitis/abscess! Feed q2 h + hand-express + correct latch; wedge-red + fever = antibiotics + continue feeding (abscess = pause that side).

★ NORCET HIT

HIT: Red-flag card at discharge: fever, foul, heavy bleed, calf pain, chest pain, blues >2 wks, baby poor suck/jaundice spread.

★ PNC PEARL: Full breasts need emptying, not resting.

► Day-4 fever + foul lochia + tender uterus. Likely + act?

Endometritis — admit + broad antibiotics + vitals + fluids.

Prolactin MAKES, oxytocin MOVES, emptying ORDERS more! Colostrum = antibody gold in teaspoons — night feeds build supply, prelacteals destroy it. Demand = supply!

Prolactin = production	Oxytocin = letdown	Night feeds build supply
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★ HORMONE / RULE — ROLE

Hormone / rule	Role
Prolactin	Makes milk; night + frequent suck boosts
Oxytocin	Letdown + bonding + involution cramps
Colostrum	Gold antibodies, small + q2 h enough
Supply rule	Empty often = make more; supplement = shrink

⚠ FATAL TRAP

Honey/water/ghutti + separated nursery 'till milk comes' starves gut flora + supply! Zero prelacteals, rooming-in, skin-to-skin, feed on cue day AND night from hour one.

★ NORCET HIT

HIT: Letdown signs = opposite leak + cramps + thirst. Growth-spurt cluster feeds (2–3 wks, 6 wks) mimic 'less milk' — weigh + pee count reassure.

★ PNC PEARL: Breasts work night shifts — let them.

► Day-2 'no milk' panic, 3 wet napkins. Counsel?

Colostrum suffices + feed q2 h + count pees/stools + weigh.

Tummy-to-mummy, ear-shoulder-hip line, WIDE mouth + areola in, lower lip out — pain means POSITION, fix it fast! Round soft nipple after = perfect latch signature!

Tummy-to-mummy line straight	Wide + areola in deep	Pain = fix position NOW
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▲ STEP — CHECK

Step	Check
Position	Whole body to breast, head free, chin to breast
Attach	Wide gape, lower lip out, chin pressed
Good signs	Rhythmic suck + swallow, no click, content baby
Fix	Break suction with finger, re-latch, try hold

⚠ FATAL TRAP

Yanking baby OFF the nipple + 'toughening' with brushes cracks + bleeds + mastitis! Break suction gently, deepen latch, air-dry + lanolin; flat nipples = syringe/hold + patience.

★ NORCET HIT

HIT: Cross-cradle + football + side-lying rotation prevent one-spot trauma. Feed 8–12×/day; one breast well-drained beats switching fast.

★ PNC PEARL: Hurt today, fixed today — never 'bear it'.

► Cracked + lipstick-flat nipple after feeds. Fix trio?

Deeper areola latch + hold change + lanolin + air-dry.

BLUES VS DEPRESSION (2-WEEK LINE)

Blues (days 3–5, tears + guilt, baby OK) fades <2 wks with sleep + support. Past 2 wks, hopeless, poor bonding, harm thoughts = DEPRESSION/psychosis — EPDS + psych NOW, never alone!

Blues <2 wks
+ support

Depression: hopeless
+ poor bond

Harm thoughts
= emergency

⚠ FEATURE — BLUES — DEPRESSION

Feature	Blues	Depression
Onset	Days 3–5	Weeks, persists >2 wks
Mood	Tearful, guilty, loves baby	Hopeless, detached, no joy
Act	Rest + help + watch	EPDS + referral + safety plan
Danger	Resolves	Harm/self-harm = emergency

⚠ FATAL TRAP

Waving off 'I want to drop the baby' as new-mom stress misses psychosis that kills TWO! Ask suicide/infanticide thoughts DIRECTLY, never leave alone, same-day psych + family safety watch.

★ NORCET HIT

HIT: EPDS ≥13 flag. Sleep protects: 4-h unbroken block via family shifts. Psychosis (confusion + delusions) = medical emergency, admission!

★ PNC PEARL: Two weeks divides weather from illness.

► 3 wks hopeless + harm thoughts. First 3?

Never alone + safety plan + same-day psych emergency.

DISCHARGE PACKAGE: COUNSEL + PPFP + DATE

**No mother leaves EMPTY-handed: EBF plan + method + 6-week date + red-flag card!
Condoms/IUCD/DMPA before discharge — fertility returns BEFORE first menses!**

EBF 6 mo plan	Method BEFORE discharge	6-w date + red card
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◆ ITEM — GIVE

Item	Give
Counsel	EBF, hygiene, rest, help, sex after 6 wks
PPFP	Condoms / IUCD / DMPA now; LAM rules if EBF
Follow-up	6 wks: mother + baby + Hb + FP + shots
Card	Red flags + helpline + MCTS ID

⚠ FATAL TRAP

Discharging 'come for FP later' returns her pregnant at 6 weeks! Fertility precedes menses — method + condoms + written date + ASHA link BEFORE she walks out.

★ NORCET HIT

HIT: Sex after 6 wks + healing + contraception + perineal comfort. Anemia recheck + deworm + IFA continue per Hb.

★ PNC PEARL: Milk plan + method + date — the discharge triple.

► Day-2 discharge, undecided on FP. Minimum package?

Condoms + method counsel + 6-w date + red-flag card + ASHA link.

NEWBORN ADAPTATION (FIRST MINUTES)

Birth = cold + air + gravity shock! Dry-cap-warm in 60 s, lungs pop with first cry, shunts reroute, gut/kidney wake (meconium + urine <24 h). HR 120–160, warm 36.5–37.5!

60-s warm-dry + cry	HR 120–160 36.5–37.5	Mec + urine <24 h each
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Σ SYSTEM — CHANGE

System	Change
Lungs	Fluid out + first cry; surfactant keeps open
Heart	Lung flow up; ductus/ovale close over hours–days
Warmth	Dry + cap + skin-to-skin; no bath 24 h
Gut/kidney	Meconium + urine each <24 h or evaluate

⚠ FATAL TRAP

Bathing + weighing + footprints BEFORE warm-dry-cry lets cold + hypoglycemia + PPHN creep! Warm chain first, procedures after stable — cold baby = flat baby!

★ NORCET HIT

HIT: Transient tachypnea (CS/no squeeze) vs RDS (preterm/grunt) — rate + recessions + O2 need triage. Surfactant + CPAP per protocol where indicated.

★ PNC PEARL: Warmth is the first resuscitation drug.

► 30 s, no cry after dry-stim. Next?

Position airway + PPV if apneic + call NRP + clock.

COMPREHENSIVE NEWBORN ASSESSMENT

Head-to-toe + vitals + weight + danger scan on DAY ONE! Anus, palate, hips, eyes, cord — the 5 most-missed spots. Poor suck, lethargy, grunt, temp swing = NICU now!

Anus+palate+hips day one	Wt + vitals + plot	Danger scan every check
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▲ CHECK — NORM / FLAG

Check	Norm / flag
Weight/length/HC	2.5–4 kg; plot + classify
Vitals	T 36.5–37.5, HR 120–160, RR 30–60
Head-to-toe	Fontanelle, palate, red reflex, hips, anus, genitalia
Danger	Poor suck, floppy, grunt, fever/cold

⚠ FATAL TRAP

Discharging without anal + palate + hip + eye check misses atresia/cleft/DDH/cataract till damage! Undress FULLY in warm room + checklist + document + counsel prints.

★ NORCET HIT

HIT: Ballard/Dubowitz dates maturity. Caput crosses sutures; cephalohematoma doesn't + jaundice watch. Single palmar crease → syndrome screen.

★ PNC PEARL: Undress fully, check five, document all.

► RR 72 + grunting + recessions. Act?

Oxygen + NICU + sepsis/respiratory workup + NPO plan.

Ten warm links + KMC + dry cord + clean eyes! Mother's chest beats every warmer for stable LBW — cap + gown + feed access + watch breathing, 24x7!

10 warm links no bath 24h	KMC = chest + cap + feeds	Cord dry, eyes clean
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★ CARE — DO

Care	Do
Warm chain	Warm room/surface/towels + cap + no bath 24 h
KMC	Skin-chest + cap + feed access + monitor
Cord	Dry only; falls 5–15 d; report redness/pus
Eyes + skin	Clean + prophylaxis; oil-free vernix keep

⚠ FATAL TRAP

Oil, ghee, cow-dung or tight binder on the cord + over-bundling visitors' colds invite tetanus/sepsis! Dry cord, handwash wall, limited handling, warm-not-hot room.

★ NORCET HIT

HIT: KMC hours/day + weight + feed volume chart. Hypothermia rewarm slow + glucose check. Vernix = keep; bathe after 24 h stable.

★ PNC PEARL: Chest beats machine for the stable small.

► 2 kg stable, mother willing. Best warmth + feed?

KMC + EBM q2 h + weight + infection watch + card.

BIRTH IMMUNIZATION + MINOR NEWBORN ISSUES

Three at birth (BCG + OPV-0 + HepB-BD) + card in hand! Sticky eye, milia, breast buds, vaginal bleed = benign + watch — but pus, poor feed, yellow palms = ACT!

BCG+OPV+HepB day one	Benign: milia, buds, bleed	Pus/poor feed/ palms = act
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◆ ITEM — ACT

Item	Act
Birth doses	BCG + OPV-0 + HepB-BD same day + card
Benign + watch	Sticky eye clean, milia, buds, pseudo-menses
Screen	Weight + hearing + thyroid heel + hips
Next	6-w Penta date + MCP + danger demo

⚠ FATAL TRAP

Vaccinating a sick/shocked/LBW<2 kg (BCG) baby on autopilot + skipping HepB-BD 'for 6 weeks' breaks protocol! Screen stability first; HepB-BD within 24 h; defer + dated return documented.

★ NORCET HIT

HIT: BCG scar check at 6 wks. OPV-0 before discharge even if later IPV planned. Vitamin K already at birth — separate from vaccines counsel.

★ PNC PEARL: Three drops and pricks today, card stamped tomorrow-safe.

► Well day-3, missed HepB-BD. Give?

Yes ASAP + document + continue schedule + counsel.

INFECTION PREVENTION + FAMILY-CENTERED CARE

Hands + breast + together! Wash wall, no sick visitors, dry cord, rooming-in, father skin-time + danger demo — family does half the nursing when taught!

**Hands + cohort
+ linen**

**Rooming + demand
feeds**

**Father + demo
+ card**

Σ GUARD — DO

Guard	Do
Hands + linen	Wash + gown + daily linen + cohort sick
Cord + eyes	Dry + clean; redness/pus = culture + treat
Rooming	Mother-baby dyad + demand feeds + KMC
Family	Father skin + bath/demo + danger card + ASHA

⚠ FATAL TRAP

Bottle + honey + 20 visitors passing the baby seeds sepsis + breaks EBF! One-card entry, handwash gate, rooming-only, discharge demo signed by TWO family members.

★ NORCET HIT

HIT: Eye pus day 2–3 = ophthalmia workup (maternal STI treat too!). Umbilical flare + fever = admit + IV antibiotics + glucose support.

★ PNC PEARL: Taught family = night-duty partner.

► Day-3 purulent eyes. Act?

Clean + topical antibiotic + maternal STI screen + follow-up 48 h.

FAMILY-WELFARE COUNSELING + BCS

Her life, her pick — cafeteria choice, never coercion! BCS: needs → show all + demo → SHE picks → how-to + side effects → return date + switch-anytime promise!

Cafeteria, not prescription	She picks + demo	Switch anytime + date
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★ BCS — ASK / DO

BCS step	Ask / do
Needs	Children, spacing, health, partner
Show mix	All methods + demo + MEC fit
She chooses	How-to + side effects + backup
Return	Date + switch-anytime + card

⚠ FATAL TRAP

Pushing IUCD/sterilization for targets + hiding side effects returns anger + discontinuation + distrust! Full disclosure + refusal respected + documented — targets never beat consent.

★ NORCET HIT

HIT: Postpartum + postabortal slots counselled BEFORE discharge. Male involvement invited, never forced. Unmet need = counsel gap, fix it.

★ PNC PEARL: Consent outlives every target.

► Two living kids, wants no more. Offer?

Permanent + LARC menu + counsel + consent + date.

LAM expires at 6 months (EBF + amenorrhea + <6 mo — ALL three)! Condoms = dual protection + EC backup; diaphragm needs fitting + 6-hour rule!

**LAM = 3 rules
+ 6-mo expiry**

**Condoms = dual
+ backup**

**Diaphragm: fit
+ 6 h**

▲ METHOD — PEARL / FAIL

Method	Pearl / fail
LAM	<6 mo + EBF + no menses; else switch
Standard days	Cycle 26–32 + beads; irregular = no
Condom	Dual STI + pregnancy; tear = EC
Diaphragm	Fitted + spermicide + stays 6 h post

⚠ FATAL TRAP

Calling 8-months + top-feeds + 'no menses' protected returns pregnancy! LAM breaks with ANY rule fail — bridge (POP/condoms) BEFORE expiry, dated switch visit fixed.

★ NORCET HIT

HIT: Withdrawal + spermicide-alone = high fail; document typical-use rates honestly. Female condom demo + practice builds use.

★ PNC PEARL: LAM's clock stops at six months sharp.

► 7 mo + supplements + amenorrhea. Protected?

No — switch to POP/condoms/IUCD today.

Time-discipline = pill success! COC same time daily + missed-1/2/3+ rules; POP 3-hour window (lactation-safe); DMPA 3-monthly + bone/weight counsel. Migraine-aura/smoker/VTE = NO estrogen!

Same time daily	Missed 3+ = EC + backup	Aura/smoker/VTE = no COC
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◆ METHOD – RULE

Method	Rule
COC	Start day 1–5; missed 1/2/3+ ladder + backup
POP	Same 3-h window; EBF-safe
DMPA/Antara	q3 mo; weight + bone + return-fertility counsel
Avoid COC	Smoker >35, aura, VTE, HTN, breast cancer

⚠ FATAL TRAP

COC to aura-migraine/smoker/VTE 'because she asked' courts stroke! MEC screen EVERY starter + yearly BP/smoking/migraine check — switch to CuT/POP fast, document choice.

★ NORCET HIT

HIT: Vomit/diarrhea pill = missed pill rules. Rifampicin + some anticonvulsants cut efficacy — backup + switch. DMPA delay 2 wks grace, then EC screen.

★ PNC PEARL: Clock, aura, smoke — three questions before COC.

► Missed 3 COCs + unprotected last night. Act?

EC now + backup 7 d + pregnancy test date + counsel.

IUCD (CuT): CHOICE TO STRINGS

CuT 380A = 10 years of forget-free cover! Interval day 2–12 or PPIUCD <48 h, sterile + consent, strings at 6 wks — heavy bleed/cramps + lost strings = check position, offer switch!

380A = 10 yrs 375 = 5 yrs	Sterile + consent + screen	Strings 6 w + red flags
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Σ STEP — DETAIL

Step	Detail
Choice	380A 10-yr / 375 5-yr; nullipara ok
Insert	Interval 2–12 d / PPIUCD <48 h; no PID/PPH
Check	Strings 6 wks; lost + doubt = USG
Remove/switch	Infection/pregnancy/menopause + new plan

⚠ FATAL TRAP

Insertion with PID/PPH/fever or blind tugging lost strings with pregnancy doubt perforates + spreads! Screen, test + USG first, strings + card + red-flag counsel every user.

★ NORCET HIT

HIT: Heavier periods expected 3 mo (NSAID + Hb watch); expulsion peak with PPIUCD — 6-w check non-negotiable. Perforation (severe pain + lost) = OT!

★ PNC PEARL: Strings felt, seat confirmed — till 6 weeks says so.

► Post-CuT flooding + cramps + Hb fall. Manage?

NSAID + Hb + position/size check + switch offer + follow-up.

PERMANENT METHODS + EMERGENCY CONTRACEPTION

Minilap/Pomeroy + NSV need consent + regret screen — vasectomy sterile only AFTER 3-month semen! EC <72–120 h ASAP (CuT <5 d best); vomit = repeat dose!

NSV: 3-mo semen
till clear

EC ASAP
<72–120 h

CuT <5 d =
best EC

★ METHOD — RULE

Method	Rule
Tubectomy	Minilap/Pomeroy; postpartum + interval options
NSV	No-scalpel; condoms till 3-mo semen clear
EC pills	<72–120 h ASAP; vomit <2 h = repeat
CuT-EC	<5 d + continues as method — gold

⚠ FATAL TRAP

Declaring NSV 'sterile from today' + unprotected tonight fathers the next baby! Condoms till azoospermia slip + date + backup EC knowledge — counsel the WAIT at consent.

★ NORCET HIT

HIT: EC needs no test first — give + start regular same cycle + follow-up bleed/pregnancy test. Regret screen: young + unstable + coerced = pause + LARC.

★ PNC PEARL: Snip today, clear in ninety days.

► Unprotected 48 h, no method. Offer + follow?

EC now + start regular + test date + counsel.

MEC (WHO ELIGIBILITY 1-4)

Four numbers decide safety: 1 free, 2 wins, 3 last-resort, 4 NEVER! Postpartum: CuT <48 h ok (then 4-w wait), POP/DMPA early ok, COC waits (6 w EBF / 3 w non) — MEC card in pocket!

1 free, 2 wins 3 last, 4 never	CuT <48 h ok then wait	COC waits: 6w/3w rule
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▲ CAT — MEANS

Cat	Means
1	Use freely, no restriction
2	Benefits outweigh — use + watch
3	Risks win — last resort + expert
4	NEVER — absolute bar

● Postpartum CuT: <48 h cat 1–2, then wait till 4 wks. POP/DMPA: early use ok. COC: 6 wks if EBF, 3 wks if not + VTE screen.

⚠ FATAL TRAP

COC on day-10 EBF 'because periods absent' clots + dries milk! Postpartum VTE + lactation rules first — POP/condoms/CuT bridge to 6 wks, then re-MEC.

★ NORCET HIT

HIT: SMOKER + age + aura + HTN push COC to 3–4 — switch, don't argue. MEC wheel/app at every FP corner beats memory.

★ PNC PEARL: Four shuts the door — memorize its residents.

► EBF day-10 requests COC. Give?

No — POP/condoms/CuT till 6 wks, then re-MEC.

ADOLESCENT & YOUTH-FRIENDLY (RKSK/AFHS)

Friend first, provider second! Alone-time + confidential card + WIFS + condoms/EC knowledge + delay-marriage + schooling — judgment closes the door, privacy opens it!

Alone-time + private card	WIFS + deworm + BMI	Delay marriage + school
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◆ NEED — ACT

Need	Act
Privacy	Alone minutes + confidential record
Nutrition	WIFS + deworm + BMI + diet
SRH	Condoms + EC + STI knowledge, no scold
Future	Delay marriage + school + scheme link

⚠ FATAL TRAP

Reporting EVERYTHING to parents or laughing at questions ends visits forever — and starts unsafe abortions! Confidential (harm-exception only) + peer educator + dated return.

★ NORCET HIT

HIT: PEER (Saathiya) + AFHS clinics + helpline 1800. Anemia + early pregnancy + GBV = the adolescent triple screen every contact.

★ PNC PEARL: Open door, shut mouth (about her secrets).

► 16-yr anemic + engaged to marry. Trio?

WIFS + delay-marriage counsel + SRH privacy + school link.

Separate FIRST, believe ALWAYS! LIVES: listen, inquire, validate, safety, support + injuries + EC/PEP + MLC evidence + shelter/181 — joint counseling with abuser NEVER!

Separate + believe	LIVES + safety plan	MLC + shelter + 181
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⚠ STEP — DO

Step	Do
Listen/inquire	Private room + exact words + believe
Validate/safety	Danger + children plan + documents
Care	Injuries + EC/PEP + MLC + sealed samples
Link	Shelter + legal + 181/112 + follow-up

⚠ FATAL TRAP

Joint 'couple counseling' with the abuser present + promising total secrecy escalates violence! See alone, keep code-word + bag ready, break secrecy ONLY for imminent lethal risk.

★ NORCET HIT

HIT: One-stop (Sakhi) centres bundle care. Document body-map + photos + chain intact. Follow-up date = lifeline, never discharge to danger without plan.

★ PNC PEARL: Belief is the first treatment.

► Assault + torn clothes + police untold. Trio?

Treat + preserve samples + inform MO/police + shelter.

Match method to LIFE, not vice versa! Newborn-spacing → LAM→POP→IUCD; no-more → permanent/LARC; emergency → EC+regular; HTN/smoker → CuT/POP — switch ANYTIME!

Spacing → LAM POP, IUCD	No-more → permanent	Risky estrogen → CuT/POP
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Σ NEED — BEST FIT

Need	Best fit
Newborn spacing	LAM → POP → IUCD ladder
No more children	Permanent / LARC + consent
Emergency	EC + start regular same cycle
Youth/unmarried	Condoms + EC knowledge + privacy
HTN/smoker/VTE	CuT / POP per MEC — no estrogen

⚠ FATAL TRAP

One-method-for-all corners + 'no switch' threats spike discontinuation + abortion! Revisit choice EVERY contact (postpartum, abortion, side-effect visit) — method follows life stage.

★ NORCET HIT

HIT: Dual method (condom + effective) for STI risk + backup. Failure talk honest: typical-use rates + EC bridge + return date on card.

★ PNC PEARL: Life changes, method follows — review every visit.

► HTN + smoker + needs 3-yr spacing. Pick?

CuT IUCD — estrogen-free + long + reversible.